

RETURN TO WORK AND WORKPLACE CONTROLS — PROPOSALS FOR CONSULTATION

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August 2026 — tabled for discussion; nothing in this document is a demand, an allegation,

or a variation of any request or notice already on foot.

PART 1 — WHAT I HAVE ASKED FOR, AND HOW I PROPOSE EACH BE DONE

1. RETURN TO WORK. Under the Proposal for Return to Work of 3 August 2026: a staged return on the pattern certified in the Employee Capabilities Checklist of 3 July 2026, with the controls set out in that proposal, the Checklist's own review date of 28 August 2026 as the first checkpoint, a support person present at the return discussion, and the plan recorded in writing.
2. PAY AND LEAVE. Corrected through the ordinary payroll machinery: the rostered shifts processed by AVAC, the leave debited since 3 July 2026 re-credited, and the codings confirmed in writing.
3. THE PSYCHOSOCIAL RISK ASSESSMENT (clause 7.2; Managing the risk of psychosocial hazards at work Code of Practice 2022). A scoping meeting with me, my union representative and any Health and Safety Representative; the hazard categories to include those identified in the Health Service's letter of 7 July 2026 together with organisational justice and job insecurity; the data sources the Code identifies (complaints, absence, turnover, EAP usage); an assessor independent of Corporate Services; a written scope and timeline; findings shared; controls selected in consultation in the order the WHS Regulation requires.
4. THE MEDICAL INFORMATION. Per my response of 3 August 2026: the Health Service confirms the questions it presses; the questions are put through the allocation in that response; costs as the Health Service has committed in writing.
5. THE PROCESS, NAMED. The instrument, steps, decision-maker and end-decision of the process I am being managed under, in writing; or, if none applies, a documented return-to-work plan agreed with me, with review dates, per the Workplace Rehabilitation Standard.
6. SEPARATION OF FUNCTIONS. Decisions concerning my attendance, duties and pay made by an officer outside Corporate Services, with the delegation recorded in writing. This is the ordinary separation; it requires no restructuring and no finding about anyone.
7. FEEDBACK (clause 7.1.11). A short written response to each work health and safety matter I have raised: received, assessed, outcome.
8. RECORDS. Copies of preserved HR policies E12 and E13, and the roster and payroll record for the twelve months preceding 3 July 2026.

PART 2 — THE SWITCHBOARD OPERATING MODEL (PROPOSED CONTROLS)

These are offered as controls for discussion in the assessment and under clause 7.1.5. Each concerns the design of the work, not any person. Implementation cost is near zero: rostering and process changes only.

A. PATIENT SAFETY AS THE DESIGN PRINCIPLE. The Switchboard activates the hospital's emergency codes and relays urgent clinical results; whether the right clinician is reached, and how quickly, depends on the Switchboard and the accuracy of the on-call information it relies on. I propose: (i) clear ownership, with a backup, for the accuracy of the on-call roster and the paging system; (ii) directory updates restored to a workable pathway with a backup when the responsible officer is unavailable; and (iii) a defined escalation path for when a department's support officer or the manager cannot be reached.

2. CONSULTATION BEFORE OPERATIONAL CHANGE. No change to Switchboard operating processes takes effect until the operators who work the board have been consulted. The Switchboard is a real-time

environment operating multiple concurrent systems; the operators hold the working knowledge of it. This is the consultation the Work Health and Safety Act already requires, applied systematically.

3. A ROTATIONAL, INTEGRATED MANAGER. The manager of Switchboard Services works a regular rostered rotation on the board — on the phones, in the room. Management of a fast-paced, direct, real-time environment is soundest when management holds direct current knowledge of it; presence also ensures supervision and support are available where the work happens. This is the standard model in emergency communications settings.

4. A COMPLAINT-RESPONSE STANDARD. Complaints and escalations sent to Switchboard management are acknowledged within one business day and substantively answered within three; where the manager is unavailable, the matter escalates automatically to the established Client Liaison pathway rather than returning to the operators' console.

5. ONE RULEBOOK FOR THE ROOM. The manager working in rotation is subject to the same frameworks as the operators — rostering rules, minimum notice, fatigue breaks, and leave processes — applied consistently. Consistency of process is itself a psychosocial control.

6. A 24/7 ON-SHIFT SUPERVISOR FUNCTION, ROTATING WITHIN THE TEAM. The design principle is that in a real-time environment, escalation is delay: a decision the board needs now cannot wait on a chain to an off-site or after-hours contact. Authority therefore follows the clock. On every shift, one rostered operator holds the shift-supervisor function — the real-time decision-maker for operational matters arising on the board — while continuing to work the board with the team. The supervisor is not a second manager: the manager holds the frameworks, administration and business-hours functions; the supervisor holds the shift, so that after hours, on weekends, and at 3am the room is never waiting on an escalation that cannot answer in time. Only matters that can properly wait are escalated to the manager in hours. The function rotates among operators trained and assessed for it, and carries a classification increment or in-charge allowance for the shifts on which it is held — a development pathway and recognition for competence within the room, with shared capability across several trained operators rather than dependence on any one person. When the supervisor makes a real-time decision, the operators present are consulted there and then — consultation at the speed of the room — and the decision is carried forward through the handover below. This is the standard model in emergency communications centres. The cost is a modest per-shift allowance, ordinarily offset by reduced turnover and absence.

8. STRUCTURED HANDOVER AND DECISION LOG. Each shift hands over to the next through a structured record: operational status, live issues, and the decisions made on shift with their reasons. Context transfers instead of evaporating at shift change — a recognised failure point in continuous operations — and decisions become consistent and auditable across the roster. The log also gives the manager, in hours, a complete account of the room's operation without interrupting it.

7. EVIDENCE-BASED ROSTERING. Rosters designed from the demand data the Switchboard already generates — call volumes by hour and day, and emergency-code activity — so that coverage matches the actual peaks; with fatigue rules (minimum breaks) built into the roster design rather than checked afterwards; published with proper notice; and with equity across operators visible on the face of the roster. The data required is already collected; this proposal only asks that the roster be built from it.

PART 3 — HOW I PROPOSE THE CONSULTATION ITSELF RUN

(a) I will attend any discussion at any time, with a support person and my union representative present.

(b) Discussions occur alongside, not instead of, written responses; anything agreed is confirmed in writing before it is treated as resolved.

(c) My participation in any discussion waives nothing and does not pause any process already on foot, unless recorded otherwise in writing by agreement.

I table this because I would rather build the fix than continue the dispute. Every item in it is capable of being implemented from existing resources, this month.

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